MERCY'S ANATOMY

Document MA-002: Official Series Synopsis & Format Engine

Canon Lock v1.1. Amends MA-001, Sections 3, 6 and 12

STATUS: LOCKED IN. This document makes the creator's Series Synopsis official canon. It also issues **Amendment A-1** to MA-001 so the foundation stays consistent with it. Where MA-001 and MA-002 disagree, MA-002 wins.

Contents

- 1. The Official Series Synopsis (creator's text)
- 2. Hospital Name Timeline by Season
- 3. The Voiceover Rule
- 4. The Academic-Year Season Model
- 5. The Episode Engine: How a Typical Episode Runs
- 6. The Season Finale Rule
- 7. Character Over Ethics, Without Breaking Realism
- 8. Amendment A-1 to MA-001
- 9. Canon Log Additions

1. The Official Series Synopsis

Creator's text, adopted as canon. Only punctuation has been lightly copyedited; nothing of substance has changed.

Mercy's Anatomy chronicles the lives of surgical interns, residents, and attendings at the fictional **Bullock Sloan Memorial Hospital** (formerly **Queens Grace Hospital**, Seasons 1-6; **Queens Grace Hudson Mercy Hospital**, Seasons 6-9; and then **Bullock Sloan Memorial Hospital**, Season 9-present), as the interns progressively evolve into seasoned doctors under the guidance of their residents, attendings, and the chief of surgery.

Each episode usually opens with a voiceover narration from **Marc-Anthony Bullock** or another season regular, often foreshadowing the episode's theme. Most seasons typically mirror the physicians' academic year, with each completed year elevating the residents to a higher level in the surgical field. The season finale often culminates in a dramatic event, such as a character's death or departure (but not always).

The series focuses on the doctors' professional lives as surgeons, but also gives significant attention to their personal lives. Character development and relationships frequently take precedence over medical ethics concerns. While the physicians perform intricate surgeries and treat their patients' illnesses, they also exhibit a competitive drive and seek recognition.

Upon arrival at the hospital each morning, residents may compete over who gets assigned a challenging case, often generating tension. A hospital superior typically assigns cases, further intensifying the dynamic between residents and their superiors. Episodes shift between the doctors' interactions with patients and their personal dynamics with colleagues. Once assigned a case, each doctor works to diagnose the patient, typically with the support of an attending physician, which usually results in surgery. Each episode typically features scenes of a surgery towards the center of the episode.

The show often portrays the doctors forming personal connections with their patients, with many cases indirectly reflecting the doctors' own personal struggles. Relationships between the doctors, whether romantic or platonic, frequently develop and create conflicts between their personal and professional lives.

2. Hospital Name Timeline by Season

Seasons	Official name	Transition note
1-6	**Queens Grace Hospital**	The original name. It is in use from the series start in 2005.
6-9	**Queens Grace Hudson Mercy Hospital**	The merger with Hudson Mercy happens **during Season 6**, so Season 6 contains both names. The exact episode and date are reserved for the creator.
9-present	**Bullock Sloan Memorial Hospital**	The renaming happens **during Season 9**, so Season 9 contains both names. The exact episode, date and reason are reserved for the creator.

Continuity note: the narrator, **Marc-Anthony Bullock**, shares a surname with the hospital's final name. That connection is **canon-flagged**, but what it is, and who 'Sloan' is, remain entirely the creator's

to define. No document may assume an answer before then.

3. The Voiceover Rule

- **The default narrator is Marc-Anthony Bullock**. The character will be created by the creator; no traits, birthday or backstory are assumed here.
- **Other season regulars may narrate** individual episodes. The narrator has to be a *season regular* in that season, not a guest.
- **The opening voiceover usually foreshadows the episode's theme**, which the patient case and the personal stories then test. Where used, the closing voiceover answers or complicates it.
- **The narrator only knows what they knew on that date** (Knowledge Tracker, MA-001 §9). A voiceover can never reveal information the narrator doesn't have yet.
- **Every episode document lists its narrator** on its title block.

4. The Academic-Year Season Model

Most seasons mirror the surgical academic year. In real 2005 US residency programs, the academic year turns over on **July 1**, after the March Match. Each completed year moves the residents up one level. The structure below is the *default progression* for the starting intern class; individual characters can deviate for story reasons (repeating a year, research years, leave, fellowships), but every deviation is logged.

Season	Starting class level	Milestones in play
1	Interns (PGY-1)	First shifts, first solo procedures, survival, cuts
2	Residents (PGY-2)	Teaching new interns, earning more OR time
3	Residents (PGY-3)	Leading cases, choosing a specialty direction
4	Residents (PGY-4)	Running services, chief resident race begins
5	Residents (PGY-5)	Chief resident, boards, fellowship and job offers
6+	Fellows / Attendings	Running the hospital, teaching, power and legacy. The Season 6 merger lands here.

Exact season start dates are reserved for the creator. Whatever date Season 1 opens on, every later promotion follows the July 1 turnover, and ages advance on real birthdays (MA-001 §8).

5. The Episode Engine: How a Typical Episode Runs

This is the synopsis translated into the 43-minute, five-act structure set in MA-001. It is the *usual* shape, not a cage; event episodes can break it.

Segment	Beat	What happens
Cold Open	Voiceover and arrival	Narrator's voiceover sets the theme. The doctors arrive at the hospital that morning, often with a dated time stamp in the 5-6 AM range.

Segment	Beat	What happens
Act 1	The assignment	A superior (resident, attending or chief) hands out cases. The residents compete for the challenging one, and tension sparks between them and their superiors. Personal storylines are set up.
Act 2	Diagnosis	Each doctor works up their patient, usually with an attending. A personal connection with the patient forms, and the case starts mirroring the doctor's own struggle.
Act 3	Surgery (the center)	The central surgery sequence, at roughly the episode's midpoint. Complications and OR conflict put both medical and personal stakes at their peak.
Act 4	Fallout	Results of the surgery. Romantic or platonic relationships collide with professional duty, and someone pays a price.
Act 5	Resolution	The standalone case closes (MA-001 §2: 35%). Serialized arcs move forward (65%) and a hook is planted.
Tag	Closing voiceover	The voiceover answers or complicates the theme. The final image is often personal: a bar, the rooftop, a locker room or home.

6. The Season Finale Rule

- The finale **often** builds to a major dramatic event, such as a character's **death** or **departure**, but **not always**. It can also be a disaster, a rescue, a proposal, a betrayal or a career-defining surgery.
- **No-Nerf compliance:** a death or departure has to be *earned* by the story. Nobody is made weaker, dumber or less skilled just to set it up.
- **Continuity:** a dead character stays dead unless the method of return was already established and is medically real. A character who departs keeps every established fact about them if they return.

7. Character Over Ethics, Without Breaking Realism

The synopsis says that character and relationships *frequently take precedence over medical ethics concerns*. That is the show's priority, and it works with MA-001's realism rule rather than against it:

- Doctors **can** bend or break rules: sleeping with colleagues, getting too close to patients, going around superiors, cutting corners for someone they love.
- **Real 2005 consequences still exist**: M&M, suspension, probation, lawsuits, the NY State Department of Health, HIPAA and ACGME violations. These consequences don't have to land every time, but when they do, they are realistic.
- The drama comes from the *choice*, so the show never pretends the rule doesn't exist.

8. Amendment A-1 to MA-001

MA-001 section	Was	Now (canon)
§3 Signature device	Lead's voiceover; voice and form reserved	Narrator is **Marc-Anthony Bullock** by default, or another season regular. Opening voiceover usually foreshadows the theme (MA-002 §3).
§6 Hospital eras	Era I/II/III, dates reserved	Seasons 1-6 Queens Grace; Seasons 6-9 Queens Grace Hudson Mercy; Seasons 9-present Bullock Sloan Memorial (MA-002 §2).
§3 Season length / structure	Season counts reserved	Seasons mirror the academic year by default (MA-002 §4). Episode counts are still reserved.
§12 Open slots	Bullock and Sloan identity	Marc-Anthony Bullock exists as the narrator (details still reserved). Sloan, and the reason for the renaming, are still reserved.

9. Canon Log Additions

ID	Established fact	Source
C-001	The hospital's names by season: QGH (S1-6), QGHMH (S6-9), BSMH (S9-present).	MA-002 §2
C-002	Marc-Anthony Bullock is the default voiceover narrator and a season regular. All other details are pending.	MA-002 §3
C-003	Seasons mirror the academic year, and residents move up a level each completed year.	MA-002 §4
C-004	Superiors assign cases each morning, and the residents compete for them.	MA-002 §5
C-005	The surgery sequence falls near the center of each episode.	MA-002 §5
C-006	Finales often bring a death or departure, but not always.	MA-002 §6
C-007	Bullock surname / Bullock Sloan Memorial link: flagged, undefined.	MA-002 §2

END OF MA-002. CANON LOCKED (v1.1).
Next up, whenever you're ready: Marc-Anthony Bullock's character sheet.